



Fiona Stanley Fremantle Hospitals Group

Policy and Procedure

School referral and attendance – Mental Health

Reference #: FSH-MENH-PRO-0007

Scope

Site	Service/Department/Unit	Disciplines
Fiona Stanley Hospital	Mental Health Service The Learning Centre	Medical, Nursing and Allied Health

Introduction

The Department of Education provides teaching staff to mental health services via the School of Special Educational Needs: Medical and Mental Health (SSEN: MMH) to deliver educational support to youth mental health patients who are of compulsory school participation age and unable to participate in their enrolled school programs.

This document outlines how the Mental Health Service works together with SSEN:MMH to support school attendance during an episode of mental health care. This includes patients who are receiving care outside the inpatient setting including Youth Hospital in the Home (YHITH) and Youth Community Assessment and Treatment Team (YCATT).

1. Terminology

Allocated clinician	Nursing or allied health staff allocated to escort patients to and from TLC and attend TLC when appropriate.
Ground access leave	Grounds access leave may be granted as: - unescorted ground access (UGA) - escorted grounds access (EGA). The need for an escort on grounds is determined by clinical need and risk.
Mature minor	A person aged under 18 years is assessed as being a 'Mature Minor' or "Gillick competent" if they have attained sufficient emotional and intellectual capacity to fully comprehend the nature, consequences and risks of a

	proposed action (e.g., a treatment decision), irrespective of whether a parent (or legal guardian) consents to it. A child who is assessed to be a mature minor may themselves consent to or decline the proposed treatment.
Mental Health Youth Unit	Inpatient mental health unit delivering an acute phase of mental health care for patients aged 16 -24years.
School of Special Educational Needs: Medical and Mental Health (SSEN:MMH)	Department of Education school providing educational support to inpatients and outpatients of compulsory school participation age (4-18 years) who are unable to participate in their enrolled school programs or need to engage with education pathways.
The Learning Centre (TLC)	School facilities for primary and secondary students located in FSH Level 2 State Rehabilitation Building. Facilities include an open plan, flexible classroom space, small office, and resource room. TLC is managed by the SSEN: MMH. Teaching staff working within TLC facilitate achievement of patient's educational goals within the mental health setting and assist with school or alternate learning program transitions via advocacy and liaison.
YCATT	Multidisciplinary Treatment Team providing time limited mental health support for 16 - 24 year olds in the clinic or community setting.
YHITH	Multidisciplinary Treatment Team providing inpatient care for a specified time period at the patient 's place of residence.

2. Policy

- SSEN: MMH and youth clinical staff will collaborate to support positive educational outcomes for young people.
- The SSEN: MMH service uses the LETS model – liaison, education, transition, and support in continuing cycles to deliver educational support.
- School sessions will provide routine through non-clinical, normalised experiences for young patients and support educational continuity during mental health care.
- Youth clinical staff will support SSEN: MMH input, promote school participation as part of the young person's routine and ward culture while an inpatient and assist with transition planning on discharge.

3. Referral process

3.1. SSEN: MMH Inclusion criteria

- All school aged mental health patients who are (or should be) engaged in education in the community may be referred for educational access and support.
- Patients whose assessment by the consultant psychiatrist/treating team, has determined that:
 - the patient is clinically able to undertake education requirements and likely to benefit
 - the patient is unlikely to be adversely affected by participation in education
 - the patient's mental state or behaviour will not impede the participation of other students attending TLC
 - the patient's individual clinical risks do not contraindicate school attendance.

3.2. Referral procedure

SSEN: MMH consent and referral forms are available on the Learning Centre's [Hub page](#) . Hard copies are also available from the Youth Unit staff office or from SSEN: MMH teachers.

3.2.1. Following the patient's admission to the mental health service a "Referral to [SSEN: MMH Teacher' form](#) will be completed for the patient.

- In the inpatient setting, the registrar will complete the form, however in other settings the referral may be completed by any clinical team member.

3.2.2. The admitting nurse or care coordinator will ensure the [SSEN: MMH Consent for Release and Exchange of Information](#) form is signed by the parent/guardian and signature witnessed

- EXCEPTION: If mature minor status is confirmed by consultant Psychiatry only the patient is permitted to sign the form.

3.2.3. Both completed forms must be emailed to the generic SSEN email ssenmmh@education.wa.edu.au or hard copy provided to SSEN:MMH staff

3.2.4. Clinicians will document form completion in the medical record.

3.3 SSEN: MMH referral management

- Upon referral receipt, SSEN: MMH teachers will conduct an initial individual patient session to assess the patient's current education pathway progress and explore the most appropriate educational goals during mental health care.

- Teaching support may commence upon referral, however teachers will not contact the enrolled school for feedback, planning or transition without a signed completed parent/guardian SSEN: MMH Consent Form.
- If it is beneficial for a young person's transition to attend their enrolled school whilst an inpatient, the Registrar or other relevant FSH MH clinical staff member will inform a SSEN: MMH teacher in a timely manner to ensure planning with the school and a positive outcome for the young person.
 - Teachers will assist with the completion of the 'Inpatient School Attendance Form'.

4. School attendance

4.1. Inpatient attendance at TLC

A Referral to SSEN: MMH Teacher form indicating 'Hospital Teaching' is required must be completed in all settings prior to a patient attending TLC for teaching engagement and support.

- The patient will not attend TLC in the first 24 hours of admission to the inpatient ward, to allow for assessment and appropriateness for referral to SSEN: MMH be determined.
 - Patients on a Patient Controlled Crisis Admission Plan (PCAP) who are known to ward and TLC staff and would benefit from early access to TLC do not have to wait 24 hours
- Prior to teaching sessions in the TLC, the SSEN: MMH teachers will identify patients who have completed an initial education session and inform the shift coordinator of potentially eligible patients to attend TLC.
- The Multidisciplinary Team (MDT) will decide daily whether individual patients are clinically suitable to attend TLC. The decision will be documented in the medical record and iSOFT, together with leave arrangements to access TLC.
- Patients will be informed at an appropriate stage of their admission that school attendance is expected, provided they are physically and mentally fit to attend.

4.2. Handover between SSEN: MMH and clinical staff

- SSEN: MMH teachers are provided an iSOFT handover sheet in inpatient settings and attend morning intake /handover meetings and MDT meetings within their allocated setting.

- The SSEN: MMH staff will attend clinical intake/handovers and relevant MDT meetings where clinical information will be shared and issues may be discussed such as
 - risk and risk mitigation in the educational setting
 - diagnostic implications related to education
 - how each patient will most effectively function in the classroom
 - ways to maximise learning.
- During the MDT meeting allied health staff will handover to SSEN: MMH results of any formal assessments/ testing and communicate appropriate changes to educational approaches where possible.

4.3. Daily Youth Unit / TLC procedure

- SSEN: MMH teaching staff will provide school sessions in TLC Monday to Friday afternoons (13:15hrs to 15:15hrs), and Wednesday and Friday mornings (10:45hrs to 12:15hrs).
- Educational support may occur on the ward in consultation with the Youth Unit clinical staff.
- During morning shift, the shift coordinator identifies an allocated clinician in consultation with the MDT to support and supervise patients attending TLC where appropriate
- The shift coordinator will provide verbal handover to the rostered teacher regarding patients attending the TLC prior to patient escort by the allocated clinician. Handover will include:
 - names of referred young people appropriate to attend TLC.
 - each patient's ground access status, i.e. escorted ground access (EGA) or unescorted ground access (UGA)
 - any risk management plans or circumstances for monitoring, including patient medical issues e.g., allergies, diabetes
 - the expected arrival time at TLC if not the routine 13:15 hrs Monday to Friday and at 10:45 am on Wednesday and Friday.
- Individual therapy sessions will not be scheduled in times of school attendance where possible
- At the end of each school session, rostered teachers will hand over any important information or patient progress to the allocated nurse or shift coordinator. Clinicians will document in the medical record.

4.4. Role of the allocated clinician attending TLC

- The allocated clinician supports teaching staff in achieving educational goals, provides positive encouragement and assists to maintain the group dynamics.
- The allocated clinician must escort the patient group to TLC, monitor the patient group throughout school sessions if required, respond to any risk issues, and return the group to the Youth Unit.
 - In the case of a single patient with UGA who wishes to return to the unit, the allocated clinician /school staff will call ahead to inform ward staff the patient is returning.
 - Patients with EGA will have the allocated clinician or another health team member provide the escort.
 - Clinicians providing constant supervision/care for an individual patient (specialling), as per medical orders, will always remain with the patient in TLC.

4.5. School liaison and handover

- On receipt of signed consent (refer to Section 3.3) the SSEN: MMH will liaise with schools on behalf of the clinical team as per the referral request (refer to Section 3.2).
- Feedback from enrolled schools will be provided to the referrer/registrar (inpatients), requesting clinician (outpatients / YHITH) or relevant MDT.
- Clinical staff will enter relevant liaison information in the digital medical record (DMR)
- SSEN: MMH will support the Youth clinician to liaise with the Principal or delegate regarding transition and risk management planning for return to school from inpatient or non-ward inpatient settings, .
- Information to be supplied by FSH Clinical staff on handover to schools may include:
 - DRG (F code) to help schools determine provision of additional appropriate supports and a medical certificate.
 - admission reason, current treatment plan (including medication that may need to be taken at school), advice for successful return to school and discharge date. Support may be provided for schools to develop their own risk management plan.

4.6. Post discharge from the Mental Health Service:

- A mental health patient of school age with complex mental health needs who is deemed unfit for school at time of discharge by the Consultant Psychiatrist can be referred to the SSEN: MMH Medical Referral Program for online teaching if the student will be absent from school for between 10 days and 10

weeks and an end dated medical certificate will be provided (see the [Referral to SSEN:MMH Teacher Form](#))

- SSEN: MMH Medical Referral Program can provide online teaching for four hours per week for a specified period.

4.7. Safety considerations

- SSEN: MMH school staff are encouraged not to wear lanyards or jewellery around their necks at any time.
- SSEN: MMH school staff will be provided with a personal duress alarm by the hospital.
- The TLC wireless phone and Youth Unit extension phone must always be accessible during school sessions
- All sharps will be safely locked away and carefully monitored when in use e.g., count in/out a minimum number of scissors for relevant activities.
- Teachers will remain alert to potential risk items in TLC environment.
- TLC exit will remain unlocked and accessible during sessions.
- If staff feel threatened, they will call a Code Black immediately.
- If a patient of concern is blocking the exit and there is imminent risk to self or other staff are to retreat into the office or most appropriate location and ensure other patients/staff are as safe as possible or removed from the area.

4.8. Clinical incident management

4.8.1. The allocated clinician and rostered teacher/s will respond proactively to any escalation in individual or group dynamics, diffusing potential issues early to prevent an incident.

4.8.2. The allocated clinician or SSEN: MMH school staff will contact the Youth Unit immediately regarding absent or missing patients or if they are concerned about:

- any escalation of risk or mental state deterioration
- taking unnecessary time/not responding while in the bathroom
- any other incident.

4.8.3. The Youth Unit will immediately provide clinical staff to manage the incident.

4.8.4. If the TLC cannot contact the Youth Unit or the Youth CNS, SSEN: MMH school staff or the allocated clinician will activate a Code Black Personal Threat alarm using the duress if required.

4.8.5. SSEN: MMH school staff will not pursue patients absconding from TLC.

4.8.6. Mental health staff will notify clinical incidents in the Datix Clinical Incident Management System with input from teaching staff as appropriate and

document in the medical record .

5. Compliance/performance monitoring

The Youth Unit Clinical Nurse Specialist will evaluate compliance with this document via routine clinical incident monitoring processes.

6. Related policy documents

- WA Health
 - [Clinical Incident Management MP 0122/19](#)
- SMHS
 - [Security: Staff Identification SMHS: 106](#)
 - [Dress Code SMHS: 52](#)
- FSFHG
 - Clinical Risk Assessment and Management (FSFH-HW-POL-0017)
 - Sharps safe handling during therapeutic activities (FSFH-MENH-POL-0024)
 - Recognising and responding to acute deterioration (FSFH- HW-POL-0033)
 - Clinical documentation (FSFH-HW-POL-0039)
 - Anticipating and reducing the risk of unpredictable behaviours (FSFH-MENH-POL-0027)
 - Code Black Mental Health (FSFH-MENH-POL-0014)
 - Patient Observation- Specialising and Close Observations (FSFH-MENH-POL-0023)

7. Related standards

- NSQHS
 - Clinical Governance
 - Partnering with Consumers
 - Comprehensive Care
 - Communicating for safety
 - Recognising and responding to acute deterioration

- Chief Psychiatrists Standards for Clinical Care
 - Consumer and carer involvement in individual care
 - Risk assessment and management

8. Authorisation

EXECUTIVE SPONSOR: Nurse Director, Service 5					
Version	Date Issued	Compiled/Revised By	Committee/Consumer Group Consulted	Endorsed By	Revision due
1	08/2024	CNS, Clinical Improvement	NMPGC	S5 SQR Committee	08/2027